SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Bevacizumab + capecitabine

Advanced colorectal cancer · NCCP 00623 · version 3a · 21-day cycle

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Patient name	DOB	Hospital no.	Date

Treatment context
This combination treats metastatic or unresectable colorectal cancer when an irinotecan- or oxaliplatin-based combination is unsuitable. Its aims may include cancer control, symptom relief and longer survival.

Schedule
Bevacizumab infusion on Day 1; capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Capecitabine
ORAL CHEMOTHERAPY

- ☐ Diarrhoea and dehydration
- ☐ Painful hands / feet
- ☐ Mouth soreness and reduced intake
- ☐ Low blood counts / serious infection
- ☐ Chest pain / heart problems
- ☐ Severe early toxicity, including DPD-related toxicity

Bevacizumab
ANTI-VEGF TARGETED ANTIBODY

- ☐ High blood pressure / urine protein
- ☐ Bleeding or blood clots
- ☐ Delayed wound healing
- ☐ Rare bowel perforation / fistula
- ☐ Rare serious neurological effects
- ☐ Infusion reaction

Confirm DPD testing, tablet directions, missed-dose and stop-and-call instructions, kidney/liver and blood monitoring, interactions, fertility and contraception. Discuss bleeding, clotting, blood-pressure, proteinuria and wound-healing risks from bevacizumab.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00623 v3a.

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Developed by Dr Paul O'Brien